

HARBORLIGHT HEALTH PLAN OF FLORIDA

MEDICAL COVERAGE GUIDELINE

Policy HHP-VEN-2025-04

Treatment of Varicose Veins and Chronic Venous Insufficiency

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Lines of business	Commercial and Medicare Advantage, as a synthetic test policy only

Description

Chronic venous insufficiency produces leg aching, swelling, skin changes, and ulceration when valves in the superficial veins fail and blood refluxes toward the foot. Initial care is compression, elevation, and walking. Procedures that close or remove an incompetent saphenous vein are used when symptoms persist and imaging confirms reflux. This policy covers selected procedures on the great saphenous vein, the small saphenous vein, accessory saphenous veins, symptomatic tributaries, and, in a narrow circumstance, perforator veins. Spider veins and treatment that does not meet the criteria below are cosmetic.

Position statement

Great or small saphenous vein

Ligation and stripping, endovenous radiofrequency ablation, endovenous laser ablation, physician-compounded or non-compounded microfoam sclerotherapy, cyanoacrylate adhesive closure, or mechanochemical ablation of the great or small saphenous vein meets medical necessity when **all** of the following are documented:

Duplex ultrasound, performed with the patient upright, shows reflux in the target saphenous vein lasting longer than 0.5 seconds;

AND

The clinical class in the CEAP classification is C2 or higher; **AND**

The vein to be treated measures at least 3 millimeters in diameter on that duplex; **AND**

At least **one** of the following indications is documented:

1. Ulceration secondary to venous stasis; **OR**
2. Two or more episodes of superficial thrombophlebitis in the limb to be treated; **OR**
3. Bleeding from a ruptured superficial varicosity; **OR**
4. Pain, swelling, itching, or burning that significantly interferes with activities of daily living, **AND** daily graduated compression for at least 6 consecutive weeks has not improved those symptoms.

Mechanochemical ablation, including ClariVein-type catheter ablation reported with CPT 36473 (first vein) and 36474 (subsequent vein), is a **covered** modality under this policy when the criteria above are met. A request that fails the criteria above is cosmetic and does not meet medical necessity.

Accessory saphenous vein

The same modalities meet medical necessity for an accessory saphenous vein when all of the saphenous criteria above are met for that accessory vein, reflux is isolated to the accessory vein or the great or small saphenous vein was eliminated at least 3 months earlier, and the accessory vein is at least 3 millimeters. Treating the accessory vein in the same session as the great or small saphenous vein is covered only when each vein independently meets criteria and the duplex shows the accessory vein emptying directly into the common femoral vein.

Symptomatic tributaries

Stab avulsion, hook phlebectomy, sclerotherapy, or transilluminated powered phlebectomy of symptomatic tributaries meets medical necessity only when performed at the same time as, or after, treatment of the saphenous vein that feeds them. Sclerotherapy of isolated tributaries without prior or concurrent saphenous treatment does not meet medical necessity.

Perforator veins

Ligation or endovenous ablation of an incompetent perforator vein meets medical necessity only when **all** of the following are true:

Duplex shows perforator reflux longer than 0.5 seconds and the perforator measures at least 3.5 millimeters; **AND**

The great, small, and accessory saphenous veins in that limb were eliminated at least 3 months before this request; **AND**

A venous ulcer is still open after that prior superficial treatment plus at least 3 months of compression; **AND**

The insufficiency is not the result of a prior deep-vein thrombosis.

Exclusion. Ablation or ligation of a perforator vein performed in the same operative session as treatment of a great, small, or accessory saphenous vein does not meet medical necessity, even when the saphenous vein itself qualifies.

Telangiectasia and other non-covered techniques

Treatment of spider veins, telangiectasia, reticular veins smaller than 3 millimeters, angiomas, and hemangiomas is cosmetic and does not meet medical necessity. CPT 36468 is not covered. Endovenous cryoablation of any vein and CHIVA (conservative hemodynamic correction) are investigational. Ultrasound guidance billed only to direct sclerotherapy of tributaries is not separately necessary.

Billing and coding

Coverage in the table is the status under this synthetic policy. A code marked investigational or non-covered does not become necessary because symptoms are present.

Code	Description	Coverage
36465	Non-compounded foam sclerosant, single truncal vein	Covered if criteria met
36466	Non-compounded foam sclerosant, multiple truncal veins, same leg	Covered if criteria met
36468	Injection of sclerosant for spider veins (telangiectasia)	Non-covered, cosmetic
36470	Injection of sclerosant, single incompetent vein other than telangiectasia	Covered if criteria met
36471	Injection of sclerosant, multiple incompetent veins, same leg	Covered if criteria met
36473	Mechanochemical ablation, first vein (for example ClariVein)	Covered if criteria met
36474	Mechanochemical ablation, subsequent vein, same extremity	Covered if criteria met
36475	Radiofrequency ablation, first vein	Covered if criteria met
36476	Radiofrequency ablation, subsequent vein, same extremity	Covered if criteria met
36478	Laser ablation, first vein	Covered if criteria met
36479	Laser ablation, subsequent vein, same extremity	Covered if criteria met
36482	Cyanoacrylate adhesive ablation, first vein	Covered if criteria met
36483	Cyanoacrylate adhesive ablation, subsequent vein	Covered if criteria met
37700	Ligation and division of the long saphenous vein at the saphenofemoral junction	Covered if criteria met
37718	Ligation, division, and stripping, short saphenous vein	Covered if criteria met
37722	Ligation, division, and stripping, long saphenous vein	Covered if criteria met
37765	Stab phlebectomy, 10 to 20 incisions, one extremity	Covered for tributaries after saphenous treatment
37766	Stab phlebectomy, more than 20 incisions, one extremity	Covered for tributaries after saphenous treatment
0524T	Endovenous catheter-directed chemical ablation with balloon isolation	Investigational

Diagnosis codes that support medical necessity when the criteria above are also met: I83.001 through I83.899 (varicose veins of the lower extremities) and I87.2 (chronic venous insufficiency).

Documentation to submit

History and physical describing symptoms, how those symptoms limit daily activities, and any ulcer, bleeding, or thrombophlebitis.

The compression log: stocking pressure, start date, adherence, and the follow-up date that establishes duration. Six consecutive weeks is the minimum when indication 4 is the one being used.

A duplex report with reflux time in seconds, vein diameter in millimeters, the vein named, and laterality.

CEAP clinical class stated in the note or the duplex.

The order, naming the procedure, CPT code, vein, and laterality. If a perforator is included, state whether it is the same operative session.

Definitions

CEAP C2 varicose veins. **C3** edema. **C4** skin changes. **C5** healed ulcer. **C6** active ulcer. C0 and C1 (no signs, or telangiectasias and reticular veins only) do not satisfy the C2-or-higher criterion.

Great saphenous vein runs from the medial foot to the common femoral vein. **Small saphenous vein** runs from the lateral ankle to the popliteal vein.

Activities of daily living include standing a work shift, walking required for the household, and sleep. Cosmetic appearance alone is not an activity of daily living.

References

The citations below are published papers named so this guideline has a bibliography. They are not an endorsement, and this policy is not those organizations' guideline.

Gloviczki P, et al. The care of patients with varicose veins and associated chronic venous diseases: clinical practice guidelines of the Society for Vascular Surgery and the American Venous Forum. J Vasc Surg. 2011;53(5 Suppl):2S-48S.

De Maeseneer MG, et al. European Society for Vascular Surgery 2022 clinical practice guidelines on the management of chronic venous disease of the lower limbs. Eur J Vasc Endovasc Surg. 2022;63:184-267.

Lurie F, et al. The 2020 update of the CEAP classification system and reporting standards. J Vasc Surg Venous Lymphat Disord. 2020;8:342-352.